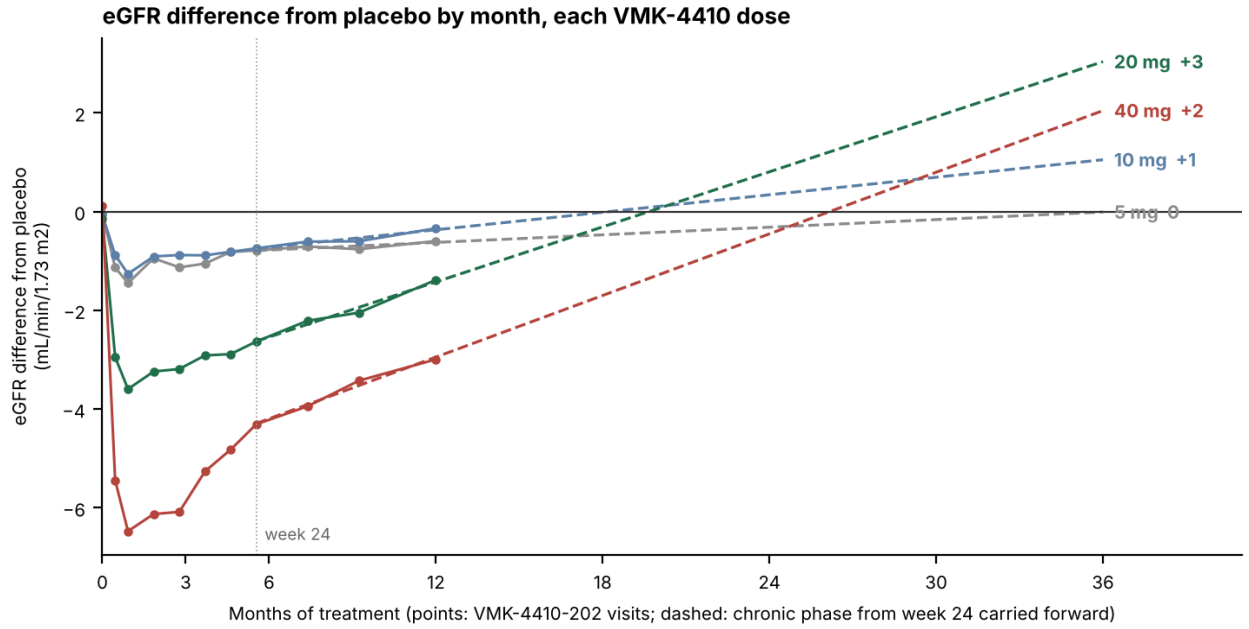
Varnmark Therapeutics A/S, Biostatistics, for the VMK-4410 end-of-phase-2 meeting package. 12 October 2026. Prepared by Nicolaj Lassen for Benedicte Laursen.

# Take VMK-4410 20 mg into VMK-4410-301: after 36 months' treatment eGFR would be 3 mL/min/1.73 m2 higher than on placebo, against 2 on 40 mg

On the phase 3 synopsis's primary estimand (every randomised patient as randomised, as if each stayed on the assigned dose, central CKD-EPI 2021 eGFR, change from baseline to the month-36 visit), 20 mg leaves eGFR 3 mL/min/1.73 m2 above placebo at month 36, 40 mg 2, 10 mg 1 and 5 mg 0. The runner-up is 40 mg, 1 mL/min/1.73 m2 behind.

| Dose | eGFR difference from placebo after 36 months' treatment (mL/min/1.73 m2) | Chronic phase, weeks 24 to 52 (difference from placebo, mL/min/1.73 m2 a year) |
|---|---|---|
| VMK-4410 20 mg | +3 | +2.2 |
| VMK-4410 40 mg | +2 | +2.5 |
| VMK-4410 10 mg | +1 | +0.7 |
| VMK-4410 5 mg | 0 | +0.3 |




## Why the chronic phase starts at week 24

Every dose lowers eGFR in the first weeks, and more so on 20 and 40 mg in patients already on a loop diuretic. The protocol kept background diuretics unchanged until the week-12 visit; on the day of the week-12, 16 or 20 visit investigators reduced or stopped the loop diuretic of 50 of the 55 patients on one on 40 mg and 35 of 61 on 20 mg (1 of 50 on placebo). Those patients recovered part of the fall over the following weeks, so the 40 mg arm's difference from placebo rose by 1.8 mL/min/1.73 m2 between weeks 12 and 24 and by 2.5 a year after that. The rise is a one-off recovery of level, not the drug's long-run effect, and the phase 3 repeats the same twelve-week rule, so its patients recover the same way. The chronic phase is fitted from week 24, when the recovery is complete, to week 52, and carried to month 36 from the level each arm settled at.

Read with the chronic phase starting at month 3, the usual split, the recovery is counted as a slope and carried for three years: 40 mg would read 5 and 20 mg 3. The week-52 difference alone, before any projection, ranks 10 mg first because it still carries the full early fall of the higher doses.

## How the figures were worked out

Central laboratory eGFR at the scheduled visits (the first sample of a visit), baseline the mean of the screening and day-1 values, each patient's change from baseline; values from samples taken after a patient's last dose set

aside for the patients who stopped study drug for good, read from the dosing record. Each arm's mean change at each visit less placebo's; a straight line through each dose's differences at weeks 24, 32, 40 and 52, read at week 156. Differences are to the nearest whole mL/min/1.73 m2.

40 mg would overtake 20 mg only if its chronic benefit over placebo were larger than 20 mg's by more than 0.7 mL/min/1.73 m2 a year.